SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Methotrexate: head and neck cancer

Recurrent or advanced head and neck cancer · NCCP 00893 · version 1 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Schedule
Methotrexate 40 mg/m² by intravenous bolus on Days 1, 8 and 15 of each 28-day cycle, until progression or unacceptable toxicity. There is no scheduled Day-22 dose. The clinician may increase to 50 mg/m² only if there has been no mucositis.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Urgent symptoms and local contact details confirmed
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Support medicines and phase changes explained
- ☐ Common, serious and lasting drug-specific risks discussed below

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Methotrexate
CYTOTOXIC CHEMOTHERAPY

- ☐ Mouth ulcers and painful swallowing (mucositis)
- ☐ Low blood counts, infection, bruising and bleeding
- ☐ Nausea, vomiting and diarrhoea
- ☐ Kidney or liver injury; accumulation with impaired kidney clearance
- ☐ Rare lung inflammation and severe skin reactions
- ☐ Fertility effects and risks in pregnancy or breastfeeding

Discuss treatment aim, expected benefit and uncertainty, alternatives and the option of declining; each medicine's risks, potentially lasting effects, required tests, urgent symptoms, fertility, contraception, pregnancy, breastfeeding and the individual support plan. Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00893 v1.

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Developed by Dr Paul O'Brien